

1. Introduction

Psychiatric hospitals play a crucial role in mental health care, providing specialized treatment for individuals experiencing acute mental health crises. In New York City, 12 psychiatric hospitals serve a diverse population with complex needs. Evaluating the performance of these hospitals is essential to ensuring high-quality, patient-centered care and improving outcomes for individuals navigating the mental health system.

This report leverages publicly available data from the Centers for Medicare & Medicaid Services (CMS) to assess hospital performance across key patient care metrics, including restraint use, seclusion rates, follow-up care, and readmission rates. By analyzing this data, we can identify strengths, uncover areas for improvement, and establish a data-driven framework to help hospitals enhance the quality of care they provide.

Using a Balanced Scorecard approach, this report assigns performance scores to NYC's psychiatric hospitals based on patient outcomes and internal care processes. While financial efficiency and operational measures are crucial for hospital success, this initial version of the scorecard focuses on CMS-available data, with the goal of expanding it in collaboration with hospitals to incorporate broader efficiency metrics in the future.

Ultimately, this report serves as both a performance evaluation and a roadmap for improvement, providing hospital administrators, mental health advocates, and policymakers with actionable insights to strengthen psychiatric care in NYC.

2. Methodology

Data Sources

This report analyzes publicly available CMS data for 12 psychiatric hospitals in New York City. The data includes performance indicators related to patient outcomes and hospital care processes, such as:

- **Readmission Rates** – The percentage of patients readmitted to a hospital within 30 days of discharge.
- **Follow-Up Care** – The percentage of patients who receive outpatient mental health care within 7 or 30 days after discharge.
- **Restraint & Seclusion Use** – The frequency of physical restraints and seclusion per 1,000 patient hours.

Balanced Scorecard Approach

To systematically evaluate hospital performance, we developed a Balanced Scorecard using the following CMS-based categories:

- **Patient Outcomes:** Readmission rates and follow-up care rates.
- **Internal Processes:** The use of physical restraints and seclusion as measures of patient-centered care and crisis management.

While financial and operational efficiencies are important factors in psychiatric hospital performance, they are not included in this version of the scorecard due to data limitations. However, this framework is designed to be adaptable and will be expanded in partnership with hospitals to incorporate additional performance metrics in the future.

Limitations & Considerations

- **Gaps in Data:** Some hospitals may not report all performance measures, which can affect comparisons.
- **Transparency & Context:** While CMS data provides valuable insights, it does not capture qualitative aspects of care, such as **staff training, patient experiences, or crisis intervention approaches**.
- **Future Expansion:** This report establishes a **data-driven foundation** for hospital evaluation, with the goal of working alongside hospitals to develop a **more comprehensive scorecard** in future iterations.

3. Key Findings & Insights

Comparison of NYC Psychiatric Hospitals

Using the Balanced Scorecard framework, each hospital was evaluated based on its performance in CMS-reported patient care metrics. The results revealed significant variations in hospital performance, with some hospitals excelling in follow-up care while others struggled with high readmission and restraint rates.

- **Top-Performing Hospitals:** Hospitals with low restraint/seclusion use and strong follow-up care had the highest scores.
- **Underperforming Hospitals:** Facilities with high restraint use and lower follow-up rates scored lower in patient-centered care measures.

Trends & Correlations

Through statistical analysis of CMS data, several important trends emerged:

1. **Hospitals with High Restraint & Seclusion Use Often Have Higher Readmission Rates**
 - While not a definitive causal relationship, the data suggests that hospitals relying on restraints and seclusion may struggle with long-term patient stability post-discharge.
2. **Follow-Up Care is a Key Factor in Reducing Readmissions**
 - Hospitals with strong outpatient follow-up rates tended to have lower readmission rates, highlighting the importance of continuity of care after hospitalization.
3. **Disparities Exist in Patient Outcomes**
 - Some hospitals consistently outperformed others in patient outcomes, raising questions about staffing, resources, and hospital policies that may impact care quality.

1. Introduction

- Brief overview of NYC psychiatric hospitals & their role in mental health care.
- Why CMS data matters in evaluating hospital performance.
- What this report seeks to uncover (**patient outcomes, hospital accountability, systemic reform**).

2. Methodology

- **Data Sources:** CMS datasets for 12 NYC psychiatric hospitals.
- **Key Metrics Used:** Readmission rates, follow-up care, restraint & seclusion use, financial efficiency.
- **Balanced Scorecard Approach:** How we structured the evaluation using patient, internal, financial, and staff metrics.
- **Limitations & Considerations:** Gaps in data, lack of transparency in some hospitals.

3. Key Findings & Insights

- **Comparison of NYC Psychiatric Hospitals**
 - Rankings based on the **Balanced Scorecard performance**.
 - Which hospitals excel, which are struggling.
- **Trends & Correlations**
 - Are hospitals with high restraint/seclusion rates also struggling with readmissions?
 - Do hospitals with stronger follow-up care have lower readmission rates?
 - What financial patterns emerge from high-performing hospitals?

4. Case Studies & Best Practices

- Highlight **top-performing hospitals** and their strategies for success.
- Discuss **underperforming hospitals** and where they need reform.
- Showcase a **national or international model hospital** that has implemented patient-centered psychiatric care.

5. Recommendations & Reform Strategies

- **Reducing Restraint & Seclusion Use:** Alternative crisis management strategies.
- **Improving Follow-up Care:** Strengthening outpatient programs, peer support, and social services.
- **Addressing Readmission Rates:** Coordinated care plans, better discharge planning.
- **Enhancing Hospital Accountability & Transparency:** Public reporting of key metrics, staff training.
- **Financial Incentives & Policy Changes:** How funding models could incentivize better care.

6. Conclusion & Call to Action

- Recap of **major findings**.
- Call for **hospital administrators, mental health advocates, and policymakers** to take action.
- Next steps for **improving psychiatric hospital care in NYC**.

7. Appendix & Data Tables (if needed)

- Include **raw hospital scores, metric breakdowns, and methodology details**.